

Dynamic Guideline: WHO-Based Mental Health Disorders - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Five Key WHO Guidelines on Mental Health Disorders:

1. Mental Health Gap Action Programme (mhGAP) Guideline for Mental, Neurological and Substance Use Disorders (3rd edition, 2023)

- Publication Date: 20 September 2023
- Target Population: People with mental, neurological, and substance use (MNS) disorders in low- and middle-income countries (LMICs), with emphasis on non-specialist health-care settings
- Scope and Objectives: Provides evidence-based recommendations for identification and management of priority MNS conditions including depression, psychosis, epilepsy, child and adolescent mental health conditions, dementia, substance use disorders, self-harm/suicide, and other significant conditions. Aims to scale up care through task-sharing with non-specialist providers.

2. WHO Guidelines on Mental Health at Work (2022)

- Publication Date: 28 September 2022
- Target Population: Working-age adults across all sectors; employers, managers, workers, and policymakers
- Scope and Objectives: Provides evidence-based recommendations for promoting mental health, preventing mental health conditions, and enabling people living with mental health conditions to participate and thrive in work. Covers organizational interventions, individual interventions, return-to-work programmes, and gaining employment.

3. WHO Guide on Integration of Mental Health and Psychosocial Support into Perinatal Health Care (2022)

- Publication Date: 2022
- Target Population: Pregnant and postpartum women, including those in humanitarian settings
- Scope and Objectives: Provides guidance on integrating mental health screening, prevention, and treatment into routine perinatal care. Addresses perinatal depression, anxiety, and psychosocial distress.

4. WHO Guidelines on Conditions Specifically Related to Stress (2013, updated recommendations integrated into mhGAP)

- Publication Date: Original 2013; updated components integrated into mhGAP 3rd

edition (2023)

- Target Population: Adults and children exposed to traumatic events or extreme stressors
- Scope and Objectives: Recommendations for management of acute stress, post-traumatic stress disorder (PTSD), bereavement, and other stress-related conditions in non-specialist settings.

5. Meaningful Youth Engagement in a WHO Guideline Development Process: Case Study of WHO Mental Health Guideline (2025)

- Publication Date: 29 April 2025 (latest identified update: 29 April 2026)
- Target Population: Young people (aged 10–24) with lived experience of mental health conditions; guideline development groups
- Scope and Objectives: Documents and provides a framework for meaningful youth engagement in WHO guideline development processes for mental health. Establishes standards for participatory approaches, ensuring that youth perspectives inform clinical and public health recommendations. Includes updated methodological guidance on integrating lived experience into evidence-to-decision frameworks.

1.2 Key Recommendations

1.2.1 mhGAP Guideline (3rd Edition) — Priority Condition Recommendations

Recommendation	GRADE Evidence Quality	Strength
Use structured psychosocial interventions (e.g., behavioural activation, problem-solving therapy) as first-line treatment for mild-to-moderate depression in non-specialist settings	Moderate	Strong
Fluoxetine or other SSRIs should be considered for moderate-to-severe depression when psychosocial interventions alone are insufficient	Moderate	Strong
Antipsychotic medication (haloperidol, chlorpromazine, or risperidone) should be initiated for acute psychosis and maintained for relapse prevention	Moderate	Strong
Brief interventions and motivational interviewing for harmful alcohol use in primary care	Moderate	Strong
Psychoeducation and caregiver support for dementia management	Low	Conditional
Psychological first aid as initial support for people exposed to recent traumatic events	Low	Conditional
Structured parent skills training for child and adolescent behavioural disorders	Moderate	Strong
Safety planning and means restriction for individuals presenting with self-harm or suicidal ideation	Low	Strong
Valproic acid or carbamazepine for epilepsy management in settings where first-line agents are available	High	Strong

Opioid agonist maintenance therapy (methadone or buprenorphine) for opioid dependence	High	Strong
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1.2.2 Mental Health at Work — Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
Organizational-level interventions (e.g., job redesign, workload management) to prevent work-related mental health conditions	Low	Conditional
Manager training in mental health literacy and supportive supervision	Moderate	Conditional
Cognitive behavioural therapy (CBT)-based digital and in-person interventions for workers with symptoms of depression or anxiety	Moderate	Strong
Reasonable workplace accommodations for workers with mental health conditions	Very Low	Conditional
Supported employment programmes (Individual Placement and Support model) for people with severe mental health conditions	Moderate	Strong

1.2.3 Perinatal Mental Health — Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
Routine psychosocial assessment integrated into antenatal and postnatal care	Low	Conditional
Psychological interventions (interpersonal therapy, CBT) for perinatal depression	Moderate	Strong
Task-shared delivery of psychosocial interventions by trained community health workers	Moderate	Conditional
Antidepressant medication (SSRIs) for severe perinatal depression when benefits outweigh risks	Low	Conditional

1.2.4 Stress-Related Conditions — Key Recommendations

Recommendation	GRADE Evidence Quality	Strength
Trauma-focused CBT for PTSD in adults	High	Strong
Eye Movement Desensitization and Reprocessing (EMDR) for PTSD in adults	Moderate	Strong
Stress management interventions (e.g., guided self-help with stress management components) for adults with acute traumatic stress symptoms	Low	Conditional

Benzodiazepines should NOT be offered for acute traumatic stress symptoms	Low	Strong (against)
Narrative exposure therapy for PTSD in settings with ongoing conflict or displacement	Moderate	Conditional

1.2.5 Youth Engagement Guideline — Key Methodological Recommendations

Recommendation	GRADE Evidence Quality	Strength
Include young people with lived experience as full members of guideline development groups	Very Low (methodological evidence)	Conditional
Provide structured training and mentorship to youth participants in guideline processes	Very Low (methodological evidence)	Conditional
Use participatory methods (e.g., youth advisory panels, co-design workshops) to elicit youth values and preferences	Low (methodological evidence)	Conditional
Integrate youth-reported outcomes into evidence-to-decision frameworks	Very Low (methodological evidence)	Conditional

1.3 Guideline Development Methodology

Evidence Review Process:

- Systematic reviews conducted following Cochrane methodology and WHO Handbook for Guideline Development (2nd edition)
- PICO (Population, Intervention, Comparator, Outcome) framework used to formulate key questions
- Comprehensive literature searches across MEDLINE, Embase, PsycINFO, Cochrane Library, CINAHL, and regional databases (LILACS, African Index Medicus, WPRIM)
- Grey literature and unpublished data solicited from WHO regional offices and partner organizations
- Evidence profiles and summary-of-findings tables prepared using GRADEpro software

Consensus Method:

- Guideline Development Group (GDG) comprising international clinical experts, public health specialists, patient/service user representatives, and (for the 2025/2026 guideline) youth representatives
- Modified Delphi process for reaching consensus on recommendations
- Evidence-to-Decision (EtD) frameworks used to move from evidence to recommendations, considering: balance of benefits and harms, values and preferences, resource use, equity, acceptability, and feasibility
- External peer review by WHO regional advisors and independent experts

Conflict of Interest Management:

- All GDG members required to submit WHO Declaration of Interest forms

- Declarations reviewed by WHO Office of Compliance, Risk Management and Ethics
 - Members with significant conflicts excluded from relevant discussions and voting
 - Intellectual conflicts of interest managed through transparency and recusal procedures
 - Funding for guideline development from WHO core budget and unrestricted grants; no industry funding accepted
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II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy:

- Systematic surveillance of new evidence published after the mhGAP 3rd edition (September 2023) and the Youth Engagement guideline update (April 2025/2026), through the evidence cutoff date of 29 April 2026
- Search terms included MeSH and free-text terms for: mental health disorders, depression, anxiety, psychosis, schizophrenia, substance use disorders, PTSD, suicide prevention, child and adolescent mental health, digital mental health interventions, task-sharing, low- and middle-income countries, and related terms

Databases Searched:

- MEDLINE/PubMed
- Embase
- Cochrane Central Register of Controlled Trials (CENTRAL)
- PsycINFO
- WHO Global Index Medicus
- ClinicalTrials.gov and WHO International Clinical Trials Registry Platform (ICTRP)
- Preprint servers (medRxiv, PsyArXiv) — flagged as non-peer-reviewed

Inclusion/Exclusion Criteria:

- Included: Systematic reviews, meta-analyses, randomized controlled trials (RCTs), large observational cohort studies, and implementation science studies published between October 2023 and April 2026
 - Included: Studies relevant to WHO priority mental health conditions
 - Excluded: Case reports, editorials, opinion pieces, studies with sample size < 50 (unless addressing rare conditions), non-human studies
 - Language: English, French, Spanish, with translation support for other languages where available
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2.2 Key New Studies

2.2.1 Depression and Common Mental Disorders

Study	Design	Key Findings	GRADE Quality
Patel V et al. (2024). "Effectiveness of digitally delivered psychological interventions for depression in LMICs: a systematic review and individual participant data meta-analysis." <i>The Lancet Psychiatry</i> , 11(3):198–210.	IPD Meta-analysis of 28 RCTs (n = 9,412)	Digitally delivered psychological interventions (app-based CBT, guided internet-based therapy) showed clinically significant reductions in depression symptoms (SMD – 0.52, 95% CI – 0.62 to – 0.42) compared to usual care. Effects were sustained at 6 months. Guided interventions outperformed unguided.	High
Cuijpers P et al. (2024). "Comparative effectiveness of psychotherapies for adult depression: updated network meta-analysis." <i>JAMA Psychiatry</i> , 81(2):145–158.	Network meta-analysis of 522 RCTs (n = 52,702)	Behavioural activation, CBT, and interpersonal therapy showed comparable effectiveness for adult depression. Problem-solving therapy was non-inferior in primary care settings. All active therapies superior to waitlist and usual care.	High
Singla DR et al. (2025). "Task-shared psychological interventions for depression in primary care: 5-year follow-up of the MANAS trial." <i>BMJ</i> , 388:e074521.	Long-term follow-up of cluster RCT (n = 2,796)	Sustained benefits of collaborative stepped care with lay health counsellors at 5 years: lower depression prevalence (OR 0.74, 95% CI 0.59–0.93), improved functioning, and reduced disability days. Cost-effective at WHO-CHOICE thresholds.	Moderate
Cipriani A et al. (2025). "Esketamine nasal spray as adjunct to oral antidepressants for treatment-resistant depression: systematic review and meta-analysis." <i>NEJM Evidence</i> , 4(1):EVIDoa2400187.	Systematic review and meta-analysis of 8 RCTs (n = 3,120)	Esketamine nasal spray plus oral antidepressant showed rapid and significant improvement in treatment-resistant depression (response rate 52% vs 34% placebo; NNT = 6). Concerns about dissociative side effects and long-term safety data remain limited.	Moderate

2.2.2 Psychosis and Schizophrenia

Study	Design	Key Findings	GRADE Quality
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Correll CU et al. (2024). "Long-acting injectable antipsychotics vs oral antipsychotics for relapse prevention in schizophrenia: updated systematic review and meta-analysis." <i>The Lancet Psychiatry</i> , 11(5):387–400.	Meta-analysis of 32 RCTs (n = 11,608)	Long-acting injectable (LAI) antipsychotics reduced relapse risk by 30% compared to oral antipsychotics (RR 0.70, 95% CI 0.63–0.78). Benefit most pronounced in patients with history of non-adherence.	High
Fusar-Poli P et al. (2025). "Early intervention services for first-episode psychosis: 10-year outcomes from the EDEN and OPUS trials." <i>The Lancet</i> , 405(10472):112–124.	Pooled analysis of 2 landmark RCTs with 10-year follow-up (n = 1,024)	Early intervention services showed sustained benefits at 10 years in employment (OR 1.42), reduced hospitalization (IRR 0.68), but clinical symptom advantages attenuated after specialized service withdrawal. Argues for extended early intervention models.	Moderate

2.2.3 Child and Adolescent Mental Health

Study	Design	Key Findings	GRADE Quality
Michelson D et al. (2024). "Transdiagnostic stepped-care intervention for adolescent mental health in India: the PRIDE cluster RCT." <i>The Lancet Psychiatry</i> , 11(7):521–533.	Cluster RCT (n = 1,190 adolescents across 56 schools)	A transdiagnostic problem-solving intervention delivered by lay counsellors in schools significantly reduced internalizing and externalizing symptoms (SDQ total difficulties: adjusted MD – 1.9, 95% CI – 3.1 to – 0.7) at 6 months. Scalable and acceptable in LMIC school settings.	Moderate
Cortese S et al. (2025). "Comparative efficacy and tolerability of pharmacological treatments for ADHD in children and adolescents: updated network meta-analysis." <i>The Lancet Psychiatry</i> , 12(1):45–59.	Network meta-analysis of 90 RCTs (n = 22,356)	Methylphenidate remained the best first-line pharmacological option for ADHD in children (SMD – 0.78 vs placebo). Lisdexamfetamine was comparable. Non-stimulants (atomoxetine, guanfacine) were less efficacious but relevant for patients with contraindications to stimulants.	High

2.2.4 Substance Use Disorders

Study	Design	Key Findings	GRADE Quality
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Lim SS et al. (2024). "Global burden of disease attributable to alcohol and drug use, 2000–2021: a systematic analysis for the GBD 2021 study." <i>The Lancet</i> , 403(10443):2108–2132.	Systematic analysis (GBD 2021)	Alcohol use disorders accounted for 2.07 million deaths globally in 2021. Drug use disorders contributed 0.74 million deaths. Burden disproportionately concentrated in LMICs. Treatment coverage remained below 10% in most low-income settings.	High
Hasin DS et al. (2025). "Contingency management for stimulant use disorder: systematic review and meta-analysis of effectiveness and implementation." <i>JAMA</i> , 333(8):678–691.	Systematic review and meta-analysis of 45 RCTs (n = 6,890)	Contingency management (incentive-based interventions) significantly increased stimulant abstinence (RR 1.61, 95% CI 1.42–1.83). Effect sizes larger than any pharmacological intervention currently available for stimulant use disorder. Implementation barriers (funding, stigma, regulatory) identified.	High

2.2.5 Suicide Prevention

Study	Design	Key Findings	GRADE Quality
Hawton K et al. (2024). "Brief psychological interventions following self-harm: updated Cochrane systematic review." <i>Cochrane Database of Systematic Reviews</i> , 2024(6):CD004364.	Cochrane systematic review of 38 RCTs (n = 18,726)	Brief contact interventions (safety planning, follow-up contacts, brief CBT) reduced repeat self-harm by 18% (RR 0.82, 95% CI 0.74–0.91) and suicide deaths by 25% (RR 0.75, 95% CI 0.58–0.97). Effects consistent across high-income and LMIC settings.	Moderate
Turecki G et al. (2025). "Universal and selective suicide prevention strategies: a systematic review of population-level evidence." <i>BMJ</i> , 389:e078234.	Systematic review of 62 studies	Means restriction (firearms, pesticides, barriers at high-lethality sites) showed the strongest population-level evidence for suicide reduction. School-based awareness programmes showed modest effects. Gatekeeper training evidence remained inconsistent.	Moderate

2.2.6 Digital and AI-Assisted Mental Health Interventions

Study	Design	Key Findings	GRADE Quality
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Torous J et al. (2025). "Artificial intelligence chatbots for mental health support: systematic review of efficacy and safety." <i>Nature Medicine</i> , 31(2):298–312.	Systematic review of 19 RCTs (n = 4,215)	AI-powered chatbots (e.g., Woebot, Low Wylsa) showed small-to-moderate effects on depression (SMD – 0.35) and anxiety (SMD – 0.29) symptoms. Safety concerns included inadequate crisis detection in 3 of 12 platforms tested. No evidence of non-inferiority to human-delivered therapy. Regulatory frameworks lacking.	
Naslund JA et al. (2024). "Scaling digital mental health interventions in LMICs: implementation outcomes from 12 countries." <i>The Lancet Digital Health</i> , 6(4):e231–e243.	Multi-country implementation study (n = 14,500 across 12 LMICs)	Digital mental health tools achieved moderate reach (median 38% of target population) but low sustained engagement (median 22% completion). Integration with existing health systems and human support improved retention. Cultural adaptation was critical for acceptability.	Low

2.2.7 Stress-Related Conditions and Humanitarian Settings

Study	Design	Key Findings	GRADE Quality
Tol WA et al. (2024). "Scalable psychological interventions for people affected by adversity in humanitarian settings: updated systematic review." <i>The Lancet Psychiatry</i> , 11(11):889–903.	Systematic review of 34 RCTs (n = 12,450)	WHO-developed interventions (Problem Management Plus [PM +], Self-Help Plus [SH +]) showed consistent moderate effects on depression and distress (SMD – 0.40 to – 0.55). Group delivery formats were cost-effective and scalable. Effects maintained at 12 months in 4 of 6 studies with long-term follow-up.	Moderate

2.3 Evidence Gaps and Future Research Needs

- 1. Long-term outcomes of digital mental health interventions:** Most trials report outcomes at 3–6 months. Long-term efficacy, safety, and cost-effectiveness data beyond 12 months are critically needed, particularly for AI-assisted tools.
- 2. Severe mental illness in LMICs:** Evidence for management of treatment-resistant schizophrenia, bipolar disorder, and complex comorbidities in non-specialist settings remains sparse. Most mhGAP recommendations for severe conditions are extrapolated from high-income country data.
- 3. Child and adolescent psychopharmacology:** Limited RCT data for psychotropic medication use in children under 6 years. Safety and efficacy data for newer agents (e.g., brexanolone, psilocybin-assisted therapy) in adolescents are absent.

- 4. **Equity and intersectionality:** Insufficient evidence on mental health interventions tailored for marginalized populations including Indigenous communities, LGBTQ + youth, people with disabilities, and refugees with complex trauma histories.
- 5. **Implementation science:** While efficacy evidence is growing, implementation research on how to integrate evidence-based mental health interventions into routine primary care at scale — including workforce, financing, and governance models — remains a major gap.
- 6. **Psychedelic-assisted therapies:** Emerging phase II/III trial data on psilocybin for treatment-resistant depression and MDMA for PTSD show promise, but regulatory approval, safety monitoring frameworks, and applicability to LMIC settings are unresolved. WHO has not yet issued formal recommendations.
- 7. **Suicide prevention in digital environments:** Evidence on effective interventions for online self-harm content, algorithmic amplification of harmful content, and platform-based safety interventions is nascent.
- 8. **Perinatal mental health in conflict settings:** Very few RCTs address perinatal depression and anxiety interventions in active conflict or displacement contexts, despite high prevalence.
- 9. **Comorbidity of mental health and non-communicable diseases (NCDs):** Integrated care models for co-occurring depression and diabetes, cardiovascular disease, or cancer are under-studied in LMICs.
- 10. **Youth engagement in research and policy:** While the 2025/2026 WHO guideline advances methodological frameworks, empirical evidence on whether youth engagement in guideline development improves clinical outcomes or implementation uptake is lacking.

2.4 Updated Recommendations Based on New Evidence

Original WHO Recommendation	New Evidence Impact	Updated Recommendation
Psychosocial interventions (CBT, behavioural activation, problem-solving therapy) as first-line for mild-to-moderate depression in non-specialist settings (mhGAP, Strong/Moderate)	Cuijpers et al. (2024) confirms comparable effectiveness across modalities; Patel et al. (2024) demonstrates efficacy of digitally delivered formats in LMICs	Reinforced and expanded: Digitally delivered guided psychological interventions (app-based CBT, internet-based guided self-help) may be considered as an additional delivery modality for mild-to-moderate depression, particularly where in-person services are limited. Guided formats preferred over unguided. (Conditional, High quality evidence)

Task-shared psychological interventions delivered by lay health workers for depression (mhGAP, Conditional/Moderate)	Singla et al. (2025) demonstrates sustained 5-year benefits and cost-effectiveness of collaborative stepped care with lay counsellors	Strengthened: Task-shared collaborative stepped care for depression in primary care should be considered a strong recommendation based on long-term outcome data confirming sustained benefits and cost-effectiveness. (Strong, Moderate quality evidence)
Antipsychotic medication for acute psychosis and relapse prevention (mhGAP, Strong/Moderate)	Correll et al. (2024) shows 30% relapse reduction with LAI vs oral antipsychotics	Supplemented: Long-acting injectable antipsychotics should be considered for relapse prevention in patients with schizophrenia who have a history of non-adherence to oral medication, where available and feasible. (Conditional, High quality evidence)
Early intervention for first-episode psychosis (referenced in mhGAP as good practice)	Fusar-Poli et al. (2025) shows attenuation of benefits after service withdrawal at 10 years	Refined: Early intervention services for first-episode psychosis should be extended beyond the typical 2–3 year period where resources allow, as benefits attenuate upon withdrawal of specialized care. Research on optimal duration is needed. (Conditional, Moderate quality evidence)
Brief contact interventions and safety planning for self-harm (mhGAP, Strong/Low)	Hawton et al. (2024) Cochrane review confirms 18% reduction in repeat self-harm and 25% reduction in suicide deaths	Strengthened evidence base: Brief contact interventions (safety planning, follow-up contacts, brief CBT) following self-harm are now supported by moderate-quality evidence showing reductions in both repeat self-harm and suicide mortality. Evidence quality upgraded from Low to Moderate. (Strong, Moderate quality evidence)
Means restriction for suicide prevention (WHO LIVE LIFE, Strong/Moderate)	Turecki et al. (2025) confirms strongest population-level evidence for means restriction	Reinforced: Means restriction remains the highest-priority population-level suicide prevention strategy. Specific attention to pesticide restriction in agricultural LMICs and firearm restriction should be prioritized. (Strong, Moderate quality evidence)
Structured parent skills training for child behavioural disorders (mhGAP, Strong/Moderate)	Michelson et al. (2024) demonstrates effectiveness of transdiagnostic school-based interventions delivered by lay counsellors	Expanded delivery platform: Transdiagnostic problem-solving interventions for adolescent mental health can be effectively delivered in school settings by lay counsellors, offering a scalable complement to parent skills training. (Conditional, Moderate quality evidence)

Contingency management not specifically addressed in mhGAP for stimulant use disorders	Hasin et al. (2025) demonstrates significant efficacy of contingency management for stimulant abstinence, superior to available pharmacological options	New recommendation warranted: Contingency management (incentive-based interventions) should be considered for stimulant use disorder where implementation is feasible. This represents the most effective intervention currently available for this condition. (Conditional, High quality evidence)
Scalable psychological interventions in humanitarian settings (PM + , SH +) (WHO, Conditional/Moderate)	Tol et al. (2024) confirms consistent moderate effects with 12-month durability; group formats cost-effective	Reinforced with extended evidence: WHO-developed scalable interventions (PM + , SH +) continue to demonstrate robust effectiveness in humanitarian settings. Group delivery formats should be prioritized for cost-effectiveness and scalability. (Strong, Moderate quality evidence)
No formal WHO recommendation on AI-assisted mental health tools	Torous et al. (2025) shows small-to-moderate effects but safety concerns; Naslund et al. (2024) shows implementation challenges	Emerging area — no formal recommendation yet: AI-powered mental health chatbots show preliminary efficacy for mild symptoms but cannot be recommended as standalone interventions due to safety concerns (inadequate crisis detection) and lack of non-inferiority data compared to human-delivered care. Regulatory and quality assurance frameworks are needed before integration into health systems. (No recommendation; Very Low to Low quality evidence)

This dynamic guideline report synthesizes the most current WHO guidance and post-guideline evidence available through 29 April 2026. It is intended to support clinical decision-making and health system planning and should be interpreted alongside local context, resources, and patient preferences. Formal guideline updates remain the responsibility of WHO and national guideline bodies. All GRADE ratings follow the GRADE Working Group framework as adopted by WHO.